Temozolomide with short-course radiotherapy, then adjuvant treatment

Temozolomide + short radiotherapy (RT) Adults older than 65 with newly diagnosed glioblastoma who are not suitable for the standard radiotherapy regimen with temozolomide, under NCCP 00461. Use the plan matched to your diagnosis and phase; do not borrow another glioma schedule.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Concurrent daily temozolomide with radiotherapy

Active medicines: Temozolomide

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/ dose	Daily during the concurrent temozolomide course, including non-radiotherapy days; confirm the written stop date National Cancer Control Programme (NCCP) V6 Table 5, p8. Start at least 30 minutes before oral systemic anticancer therapy (SACT) (p4). Confirm stop date, delayed-emesis cover and rescue plan; review QT risk, liver function, constipation and interactions.
Pneumocystis jirovecii pneumonia (PJP) prevention	Complete the local prophylactic agent, dose and schedule	Required throughout concurrent temozolomide/radiotherapy; review continuation after radiotherapy (RT) Continue when clinically indicated for lymphopenia/steroid exposure. Do not stop automatically at the end of RT; confirm allergy, renal function, interactions and pharmacy plan.

Additional phase-specific support

- Temozolomide has moderate-to-high oral emetic potential. Take the prescribed anti-sickness plan before chemotherapy and use rescue medicine as instructed. Ask what to do if nausea prevents taking medicines or fluids.
- PJP-prevention medicine is required during temozolomide with radiotherapy in this NCCP regimen. Your team supplies the prescription and decides how long to continue, including if lymphocyte counts remain low. Do not stop it just because radiotherapy has finished.
- Anti-sickness medicines and other medicines can worsen constipation. Ask for your bowel plan, and call for persistent vomiting, severe constipation or difficulty drinking. Do not double a dose after vomiting.
- If prescribed, keep steroids and seizure medicines on separate written schedules. Chemotherapy rest days do not tell you when to stop them. Ask the pharmacist to check interactions and explain any dose changes.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00461 v3 · p4 · NCCP oral-SACT antiemetics V6: Table 5 p8; timing p4; interactions p11

NCCP regimen-specific support · Sources checked 2026-11-05 · SACTCheck v0.84.38 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Adjuvant temozolomide Days 1–5 every 28 days

Active medicines: Temozolomide

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/dose	Before temozolomide on Days 1-5; confirm the prescribed stop date and delayed-emesis cover National Cancer Control Programme (NCCP) V6 Table 5, p8. Start at least 30 minutes before oral systemic anticancer therapy (SACT) (p4). Confirm stop date, delayed-emesis cover and rescue plan; review QT risk, liver function, constipation and interactions.

Additional phase-specific support

- Temozolomide has moderate-to-high oral emetic potential. Take the prescribed anti-sickness plan before chemotherapy and use rescue medicine as instructed. Ask what to do if nausea prevents taking medicines or fluids.
- Continue or review Pneumocystis jirovecii pneumonia (PJP) prevention according to lymphocyte recovery, steroid exposure and the specialist plan. Starting five-day adjuvant cycles does not itself authorise stopping prevention prescribed during radiotherapy.
- Anti-sickness medicines and other medicines can worsen constipation. Ask for your bowel plan, and call for persistent vomiting, severe constipation or difficulty drinking. Do not double a dose after vomiting.
- If prescribed, keep steroids and seizure medicines on separate written schedules. Chemotherapy rest days do not tell you when to stop them. Ask the pharmacist to check interactions and explain any dose changes.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00461 v3 · p4 · NCCP oral-SACT antiemetics V6: Table 5 p8; timing p4; interactions p11

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